1. KPI Definition

1.1 Purpose

The KPI definition sheet establishes the key performance indicators used to monitor hospital operations, financial exposure, procedure utilization, patient activity, and resource allocation.

It defines what each KPI measures, how it should be calculated, and how management can use it to evaluate hospital performance.

The values below are taken from the dashboard screenshots. Where the precise underlying calculation is not established by the screenshots, the definition is presented as a proposed calculation requiring confirmation against the data model.

1.2 Financial and Operational KPIs

KPI	Definition / calculation	Dashboard value
Total Encounter Claim Cost	Sum of the total claim-cost field across encounters.	$101.514M
Total Procedure Base Cost	Sum of the procedure base-cost field.	$105.52M
Total Payer Coverage	Sum of the payer-coverage amount displayed in the dashboard.	$31.10M
Total Base Encounter Cost	Sum of the base encounter-cost field.	$3.24M
Total Encounters	Count of encounter records or distinct encounter IDs, depending on the model.	27,891
Total Procedures	Count of procedure records or distinct procedure IDs, depending on the model.	47,701
Average Visit Duration	Average of the visit-duration field.	0.18

1.3 Payer Coverage KPIs

KPI	Definition / calculation	Dashboard value
Covered Encounters	Number of encounters classified as covered.	14,310
No-Cover Encounters	Number of encounters classified as no cover.	13,590
Covered Encounter Percentage	Covered encounters ÷ total encounters × 100.	51.29%
No-Cover Encounter Percentage	No-cover encounters ÷ total encounters × 100.	48.71%

Management relevance: These KPIs help management monitor the distribution of encounters by coverage classification and identify opportunities to improve insurance verification and patient financial processes.

1.4 Patient Flow KPIs

KPI	Definition / calculation	Dashboard value
Encounters Under 24 Hours	Number of encounters classified as under 24 hours.	Approximately 27,830
Encounters Over 24 Hours	Number of encounters classified as over 24 hours.	Approximately 59
Percentage Under 24 Hours	Encounters under 24 hours ÷ total encounters × 100.	99.79%
Percentage Over 24 Hours	Encounters over 24 hours ÷ total encounters × 100.	0.21%

Management relevance: These KPIs help monitor patient flow and identify encounters that may require additional investigation because of longer service duration.

Note: The encounter counts above are approximate calculations based on the displayed percentages and total encounters. They should be confirmed using the underlying records.

1.5 Procedure and Encounter Utilization KPIs

KPI	Definition / calculation	Dashboard observation
Encounters by Encounter Class	Count of encounter IDs grouped by encounter class.	Ambulatory is highest, at approximately 44.95%.
Encounter Cost by Class	Sum of total encounter claim costs grouped by encounter class.	Ambulatory contributes approximately $36.28M.
Procedure Cost by Description	Sum of procedure base costs grouped by procedure description.	Electric Cardioversion contributes approximately $35.82M.
Average Procedure Base Cost	Average of procedure base cost grouped by procedure description.	Administer ICU is the highest displayed procedure.
Encounters by Year	Count of encounter IDs grouped by year.	Activity varies, with notable peaks around 2014 and 2021.
Procedures by Year	Count of procedure-related encounter IDs grouped by year.	The highest displayed value occurs around 2014.

Management relevance: These indicators help identify high-demand services, major cost contributors, and areas where staffing, equipment, and financial resources may need to be reviewed.

1.6 Demographic KPIs

KPI	Definition / calculation	Dashboard value
Procedure Base Cost by Male Patients	Sum of procedure base costs for male patients.	$55.42M (52.52%)
Procedure Base Cost by Female Patients	Sum of procedure base costs for female patients.	$50.10M (47.48%)

Management relevance: Demographic KPIs provide insight into differences in procedure-cost distribution and support further investigation of healthcare utilization patterns.